Verification of Death

- Resuscitation has been stopped (death declared)
- An Expected or Unexpected Death has occurred
  - An obvious deceased body has been found

Verify Death
Using Criteria Outlined

Circulatory
- No pulse present
- No heart sounds
- Asystole on 4 lead ECG

Respiratory
- No respiratory effort
- No chest sounds

Neurological
- Pupils unreactive to light
- No reaction to painful stimuli

Inform Dispatch
Request for police

Record Death in Patient Care Record (PCR)

Complete Verification of Death Record Form

Provide initial support to family/carers

Wait for Police Arrival and Follow Police in relation to human remains
See discussion section

It is not uncommon for there to be occasional spontaneous gasping sounds or occasional intermittent audible heart sounds soon after an expected death.
If this occurs:
- The patient should be left for 15minutes and the full procedure repeated.

Response to Life Threatening Call Required?

Inform Dispatch if:
- Suspicious death
- Body in public place or
- Body unaccompanied by responsible adult

Required by police to stay on-scene?

YES → Inform dispatch and follow dispatch instructions

NO → Respond to life Threatening Call immediately

Patient Care Goals

1. Accurately verify death to confirm that no further resuscitation efforts are required.
2. Ensure effective communication with Dispatch and other Emergency Services
3. Ensure appropriate documentation is completed.

Patient Inclusion & Exclusion Criteria

Inclusion Criteria

- Patients presenting with clinical signs suggestive of irreversible death, including:
    - Decomposition
    - Obvious rigor mortis
    - Obvious pooling (hypostasis)
    - Incineration
    - Decapitation
    - Injuries totally incompatible with life
- Patients requiring confirmation of death after resuscitative efforts have been completed and death has been declared (See Starting, Stopping or Transferring CPR CPG).

Exclusion Criteria

- Unconscious patients who do not meet the inclusion criteria listed above.

Key Patient Assessment Points

1. **Termination of Resuscitation Efforts (CPR)**
    - Verify unresponsiveness by assessing for no response to verbal or painful stimuli (AVPU: U).
    - Confirm absence of spontaneous breathing or presence of agonal gasps for at least 60 seconds.
    - Confirm absence of pulse (carotid or femoral) for at least 60 seconds.
    - 4-lead ECG showing asystole for at least 60 seconds.
    - Confirm absence of heart sounds for at least 60 seconds.
    - Confirm absence of neurological function (pupils non-reactive to light).
    - Note time of death declared
2. **Definitive Signs of Irreversible Death**
    - Check for rigor mortis (stiffening of muscles, typically onset 2-6 hours post-death).
    - Assess for dependent lividity (pooling of blood in dependent areas, appearing as purplish discoloration).
    - Identify catastrophic injuries incompatible with life (e.g., decapitation, incineration, decomposition or massive crush injuries).
3. **Environmental and Contextual Assessment**
    - Note scene conditions (e.g., body position, evidence of prolonged downtime).
    - Identify any suspicious circumstances (e.g., trauma, ligature marks) requiring police involvement.

Patient Management & Interventions

1. **Verification of Death**
    - Confirm signs of Death:
        - Verify unresponsiveness by assessing for no response to verbal or painful stimuli (AVPU: U).
        - Confirm absence of spontaneous breathing or presence of agonal gasps for at least 60 seconds.
        - Confirm absence of pulse (carotid or femoral) for at least 60 seconds.
        - 4-lead ECG showing asystole for at least 60 seconds.
        - Confirm absence of heart sounds for at least 60 seconds.
        - Confirm absence of neurological function (pupils non-reactive to light).
        - Note the time of death declared
2. **Communication**
    - Inform dispatch of death declaration and request black ambulance (deceased person ambulance).
    - Notify dispatch if:
        - Any suspicious deaths or circumstances and request police assistance.

        - Body is in a public place
        - Body is unaccompanied by a responsible adult.
    - Provide compassionate communication to family, explaining the verification process and next steps.
3. **Post-Verification Management**
    - Suspicious Circumstances
        - Preserve the scene for police or coroner investigation if suspicious circumstances are present.
        - Avoid disturbing the body or surroundings unless necessary for safety or death verification.
    - Informed by dispatch that a life-threatening call is waiting
        - Ensure dispatch is aware of any special circumstances (e.g. police not yet on scene or body is in a public place).
        - Inform dispatch if directed by police to remain on-scene.

Patient Safety Considerations

- None noted.

Clinical Insights & Additional Guidance

- Do not transport a body in an ambulance vehicle unless:
    - The body is recently deceased and in a public place where public concern may be caused if it is left there.
    - The emotional circumstance associated with the death causes severe distress for the family.
    - Prior to the transport an arrangement is in place to accept the body at the destination (coordinate and confirm with dispatch prior to loading the body onto the ambulance stretcher).
    - As instructed by higher management

Documentation

- Identification: Full name, age, gender of the patient.
- Mechanism / Medical Complaint: Context of arrest or condition leading to assessment (e.g., collapse, trauma, found unresponsive).
- Injuries / Information: Obvious signs of irreversible death (e.g., decomposition, rigor mortis, decapitation), suspicious marks, last known alive.
- Signs: Confirmed absence of breathing, pulse, heart sounds, and neurological response; 4 lead ECG strip showing asystole (≥60 seconds), if agonal ECG wait until flat line then take copy of the strip.
- Treatment:
    - Time of arrest (if known).
    - Time CPR was stopped (if applicable).
    - Time of death declaration.
    - Any interventions performed prior to verification (e.g., airway management, IV attempts).
- Allergies: Not applicable unless relevant history is provided.
- Medications: List any found medications that may indicate cause of death or chronic illness.
- Background: Medical history, living situation, and family witness statements if available.
- Other:
    - Scene description, presence of bystanders, police involvement, emotional distress observed.
    - If transported, reason and confirmation of receiving facility.
    - Communication with dispatch and coordination steps taken.
    - Accurate timestamp of each assessment point (e.g., ECG asystole start, time of death declaration).
    - Avoid unapproved abbreviations.
    - Clearly record rationale if suspicious death is suspected or police involvement initiated.

Key Performance Indicators

1. Ensure documentation includes confirmation of all vital sign absences, ECG documentation, and time of death declaration.

References

1. American Heart Association 2020, *2020 American Heart Association Guidelines for Cardiopulmonary Resuscitation and Emergency Cardiovascular Care*, American Heart Association, Dallas.
2. Ambulance Victoria 2023, *Clinical Practice Guidelines for Ambulance and MICA Paramedics*, Ambulance Victoria, Melbourne.
3. Joint Royal Colleges Ambulance Liaison Committee 2019, *JRCALC Clinical Guidelines 2019*, Class Professional Publishing, Bridgewater.
4. National Association of State EMS Officials 2022, *National Model EMS Clinical Guidelines*, Version 3, NASEMSO, Falls Church, VA.
5. Ontario Ministry of Health and Long-Term Care 2020, *Advanced Life Support Patient Care Standards*, Version 4.9, Emergency Health Services Branch, Toronto.

Neurological